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3-12-2024

Project #49 Transplant Psychology Post-Liver Transplant Follow-up Clinic

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Recommended Citation

Segal, Antú, "Project #49 Transplant Psychology Post-Liver Transplant Follow-up Clinic" (2024). *Quality Expo 2024*. 49.

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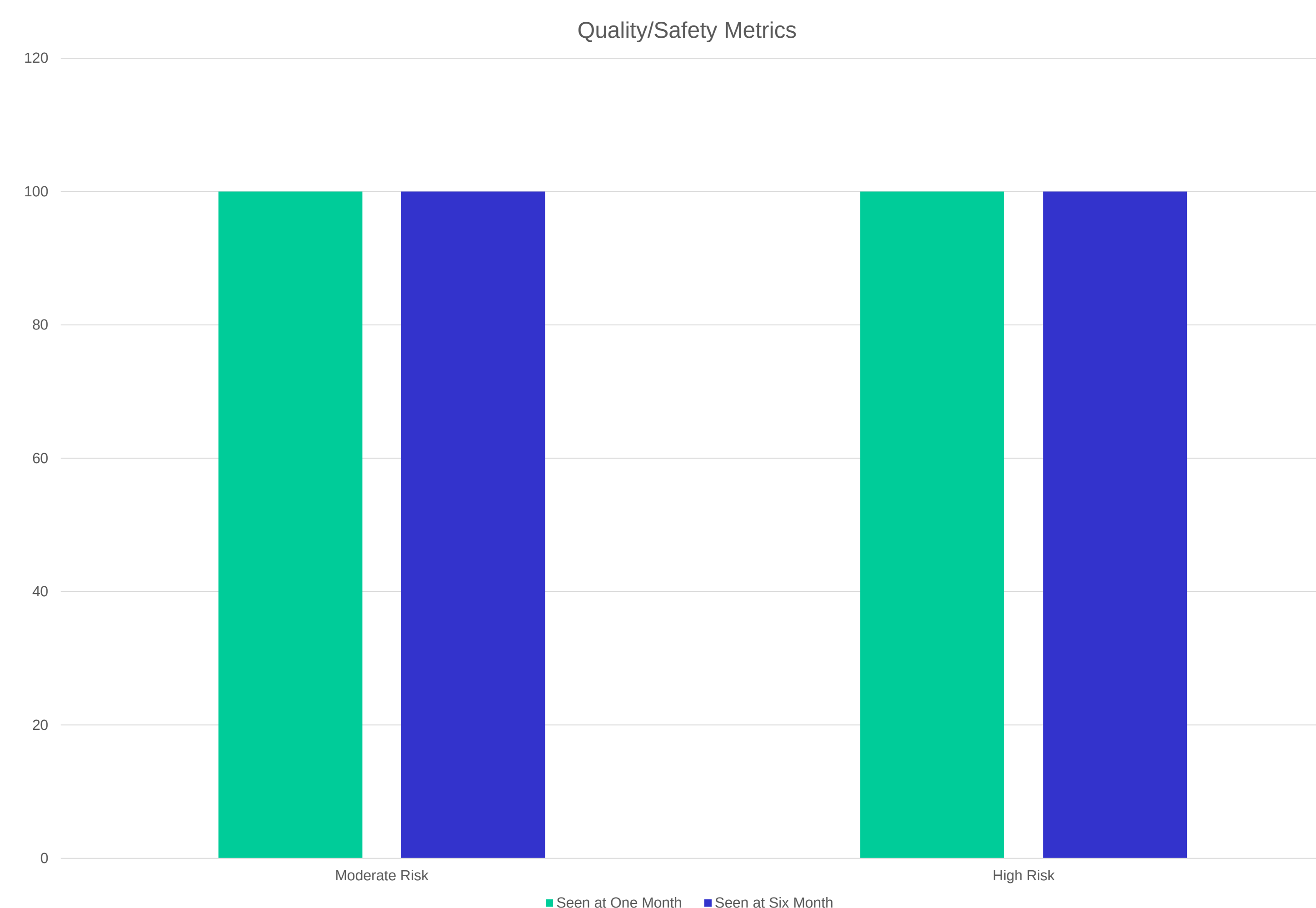
Abstract

- Liver transplant is a life-saving option for patients with end stage liver disease¹
- The journey to transplant and post-liver transplant is complicated and can be a challenge to patients and their families²
- The prevalence and connection between medical, psychological, and social factors that impact post-liver transplant patients warrants further investigation³
- Routine follow-up of post-liver transplant patients has helped in identifying and mitigating risks that impact surgical outcomes⁴
- **AIM:** Creation of a new health psychology post-liver transplant follow-up clinic pilot to address gap in treatment for post-liver transplant patients

Methods

- **Plan:** To address the lack of scheduling patients for follow-up post-liver transplant with health psychology. The current condition: Post-liver transplant patients are not routinely seen by health psychology. The root cause: Lack of awareness in post-liver transplant team of the health psychologist's bandwidth and clinical role. Hypothesis: If the clinical role of the health psychologist is explained to the post-liver transplant team, then the creation of a new clinic to see post-liver transplant patients is feasible. Change behavior: To improve workflow, post-liver transplant patients will be seen routinely, and the health psychologist will be able identify and mitigate psychosocial risks to improve surgical outcomes.
- **Do:** Rolled out a new clinic pilot for one year. Collected data to demonstrate that the health psychologist was able to see post-liver transplant patients within the intended timeframe. Data was collected by liver transplant quality/safety representatives (Annette Feeney and Anne Malilay) as part of our liver transplant team metrics. The metric was to analyze if patients identified as moderate or high risk for relapse post-liver transplant were being seen at one- and six-months post-liver transplant by the health psychologist.

If psychosocial factors that impact the post-liver transplant outcomes negatively are assessed and identified routinely, then liver transplant teams can mitigate risk and proactively guide patients and their families to better outcomes post-liver transplant by making appropriate treatment recommendations.



Methods Continued

- **Check:** At quarterly reviews of metric data, health psychologist was meeting metric at 100%. Analyzed data and evaluated the effects of the new clinic pilot. Reviewed additional feedback from the post-liver transplant team and liver transplant leadership regarding scheduling.
- **Act:** New clinic was designed to align with the existing transplant hepatology clinic on K16. Collaborated with post-liver transplant team and liver transplant leadership to understand the gaps in psychological treatment. Standardized and shared new health psychology clinic with the post-liver transplant team/leadership for feedback. Continually monitoring and adjusting to the needs/feedback of the post-liver transplant team and liver patients/families. Ongoing PDCA cycles for continued refinement of new workflow.

Conclusion

- Post-liver transplant patients at moderate and high risk for relapse are seen routinely at one- and six-months post-discharge from hospital by health psychology
- Health psychologist assesses for depression, anxiety, medication management/adherence, social support and relapse on substance use
- Treatment recommendations are made based on assessment to mitigate psychosocial risk and improve surgical outcomes

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